NCCP 00237 · clinician draft

Dabrafenib alone — advanced melanoma

The treatment this guide covers This is dabrafenib monotherapy for advanced melanoma. Do not import the food, heart-testing, fertility or duration instructions of a different BRAF/MEK combination. The NCCP fever table states 38.5°C, while current EMA product information states interruption at 38°C for monotherapy; this guide uses the earlier current-product safety trigger and the team must reconcile the written plan.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Dabrafenib alone — advanced melanoma · support plan

Active medicines: Dabrafenib

Emetogenic risk: Minimal to low per NCCP 00237 v6; local policy

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- NCCP classifies dabrafenib as minimal-to-low emetogenic risk; anti-sickness treatment follows local and individual need. Agree a written fever plan and keep a thermometer and actual urgent-contact number. The team may prescribe fever treatment and hydration advice after assessment; fever medicine is not a substitute for contacting them. Anti-sickness treatment is selected individually.
- Monthly blood count, kidney and liver tests, and monthly skin review. Repeat ECG when clinically indicated. NCCP also describes head-and-neck review every three months and other surveillance according to the individual assessment; this is separate from the cancer-response scan plan.
- If your temperature is 38°C or higher, stop dabrafenib and contact the oncology team immediately under the current product information; do not restart on your own. Also call for shaking chills, dizziness, reduced urine, severe abdominal pain, eye pain or changed vision, or a blistering or spreading rash. Infection and medicine-related fever both need consideration.
- Use effective contraception during dabrafenib alone and for two weeks after the last dose. Hormonal contraception may be less reliable; agree an effective alternative. A longer interval applies if another medicine requires it, so use your actual prescribed plan. Discuss fertility and breastfeeding before treatment.
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Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00237 v6 · NCCP 00237 v6 supportive-care section; current product information for medicine-specific precautions

Sources checked 2026-11-06 · SACTCheck v0.84.42 · Regimen-level drafts; prescriber and oncology-pharmacy verification required